INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Carboplatin and paclitaxel · lung cancer

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Treatment stages
2
Explained in the treatment reference

Regimen source
NCCP 00304
Version 8

Both medicines on Day 1, every 21 days (three weeks), for four planned cycles.

Treatment stages

01 **Four chemotherapy cycles**
Every three weeks

02 **Review and next plan**
After the course

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call your oncology team immediately for fever, shivering, sudden illness, new breathing difficulty, chest pain, unusual bleeding, severe abdominal pain or inability to drink. Report new numbness, weakness or hearing change promptly. Follow the unit's emergency instructions and do not wait for the next visit.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Three medicines. Different jobs.

After surgery, treatment aims to reduce the chance of lung cancer returning. For advanced lung cancer, it aims to control cancer and help symptoms. Your team confirms which aim applies to you.

CHEMOTHERAPY

Paclitaxel

Taxane chemotherapy



Disrupts microtubules and cell division

Paclitaxel stabilises microtubules, structures cells use during division. It can slow cancer growth but also harm healthy nerves, blood-forming cells and hair follicles. Both medicines are planned on Day 1 of each of four cycles.

Microtubules → Division disrupted → Cancer cells affected

What to watch for

Tell the nurse immediately about infusion symptoms. Report numbness, tingling, fever, painful muscles, mouth sores or diarrhoea.

CHEMOTHERAPY

Carboplatin

Platinum chemotherapy



Damages deoxyribonucleic acid (the cell’s genetic material) (DNA)

Carboplatin forms links in DNA that hinder copying and repair. Healthy tissues can also be harmed. Its dose depends on target exposure (AUC) and measured or estimated kidney function; AUC is not a fixed number of milligrams. Both medicines are planned on Day 1 of each of four cycles.

DNA → Platinum links → Copying disrupted

What to watch for

Report fever, bleeding, nausea, hearing changes or numbness. Tell the nurse immediately about possible allergy during the infusion.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

What the evidence means

The adjuvant CALGB 9633 study did not show a statistically significant survival benefit for the whole stage IB group in its mature analysis. An exploratory large-tumour subgroup suggested benefit. Advanced-disease trials support platinum-doublet activity but do not turn adjuvant results into an advanced-cancer prognosis.

Benefit depends on the cancer stage, treatment aim, fitness and alternatives. Trial averages are not a prediction for you.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Treatment → review → next steps

Treatment stages

01 **Four chemotherapy cycles**
Every three weeks

02 **Review and next plan**
After the course

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

EVERY THREE WEEKS

Four chemotherapy cycles

1 2 3 4

Both medicines on Day 1, repeated every 21 days for four planned cycles, unless cancer progresses or side effects require a change. This standalone plan does not include nivolumab or radiotherapy.

AFTER THE COURSE

Review and next plan

Review

Your team reviews recovery and the next steps. Scans or other treatments depend on the cancer setting; they are not automatically scheduled by this guide.

Your team confirms each stage and discusses any changes with you.

BASELINE → TREATMENT → FOLLOW-UP

Treatment and checks

Relative stages; each cycle is planned 21 days apart but may be delayed. No fixed scan or echo interval is implied.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00304 v8 · official regimen

THE WHOLE TREATMENT JOURNEY

Carboplatin and paclitaxel · lung cancer: treatment and checks

Standalone chemotherapy · adjuvant or advanced non-small-cell lung cancer (NSCLC)

Both medicines on Day 1, repeated every 21 days for four planned cycles, unless cancer progresses or side effects require a change. This standalone plan does not include nivolumab or radiotherapy.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Preparation	Cycle1	Cycle2	Cycle3	Cycle4	Review
Treatment NCCP 00304	Agree aim and consent	Both drugs on Day 1	Repeat if appropriate	Repeat if appropriate	Final planned cycle	Separate next plan
Blood and kidneys NCCP 00304	full blood count (FBC), renal, liver; verify GFR	Review before dose	Repeat before dose	Repeat before dose	Repeat before dose	Follow abnormalities
Symptoms NCCP 00304	Nerves/ hearing baseline	Allergy and symptoms	Nerves, hearing, blood effects	Review cumulative effects	Review cumulative effects	Manage persisting effects
Scans and decisions NCCP 00304	Stage and team plan	No universal scan date	Individual plan	Individual plan	Individual plan	Consultant agrees monitoring

What each check is for

1 **Preparation**

Before first dose

Confirm the treatment aim and suitability

Ask / **act:** Confirm consent, organ function, pregnancy advice and contacts.

2 **Before each cycle**

BeforeDay 1

Safety can change

Ask / **act:** Review blood results, symptoms and premedication.

3 **End of course**

Afterfour cycles

Agree next steps

Ask / **act:** Review recovery and individual disease monitoring.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Before the visit

Confirm blood tests, premedication instructions, transport and urgent contacts.

STEP 2

Time in the unit

Paclitaxel is given into a vein over three hours, followed by carboplatin over 30 minutes. Blood review, preparation, prevention medicines and observation make the visit longer.

STEP 3

Access

Treatment uses a suitable intravenous (IV) line. A port may be considered individually; no take-home infusion pump is part of this regimen.

STEP 4

At home

Take allergy-prevention and anti-sickness medicines exactly as prescribed. The team combines steroid instructions into one plan; do not add leftover tablets or miss a premedication without telling the team.

STEP 5

Review

Confirm the next appointment and individual scan plan before leaving.

Visits and tests

Before each cycle

Blood count, kidney and liver tests before starting and before each cycle. Tell the team about numbness, tingling, hearing change, infection or bleeding before every treatment. Scan timing follows your individual plan.

After four cycles

The team reviews response or recurrence surveillance and recovery; further treatment needs a separate plan.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Your 21-day cycle.

Timing is a general guide; symptoms may start or stop at other times.

Treatment or check	Timing in this view
Paclitaxel then carboplatin	 **Day 1** One treatment visit
Symptoms can occur any day	 **Days 1–21** No guaranteed safe or recovery days

Relative cycle days, not guaranteed symptom or recovery dates.

The next cycle starts 21 days after this one, if your team confirms it.

> Call your oncology team immediately for fever, shivering, sudden illness, new breathing difficulty, chest pain, unusual bleeding, severe abdominal pain or inability to drink. Report new numbness, weakness or hearing change promptly. Follow the unit’s emergency instructions and do not wait for the next visit.

Between-treatment notes

Day 1: Treatment day

Paclitaxel is given into a vein over three hours, followed by carboplatin over 30 minutes. Blood review, preparation, prevention medicines and observation make the visit longer. Tell staff immediately about flushing, rash, chest or throat tightness, breathing difficulty or feeling faint.

Day 2–7: Early days at home

Follow your prescribed anti-sickness plan. Aches, nausea, tiredness and bowel changes can occur. Call if symptoms are uncontrolled or you cannot drink; fever or sudden illness needs urgent advice.

Day 8–14: Keep watching for symptoms

Low blood counts can increase infection or bleeding risk. The timing varies between people. Call urgently for fever, shivering, sudden illness or unusual bleeding; do not wait for blood tests.

Day 15–21: Review before the next cycle

Symptoms may improve or persist. Describe the worst symptoms since treatment, not just how you feel today. Confirm blood tests and the next appointment. Report numbness, weakness or hearing changes before treatment.

Support around the medicines

1 Drinking and symptoms

Ask for help with poor intake, vomiting, diarrhoea or reduced urine. Follow your individual fluid advice rather than a fixed target.

2 Practical help

Plan transport if medicines make you drowsy. Bring your medicine list, symptom record and treatment card.

Your treatment and its aim

Which setting?

NCCP 00304 a/b: selected adjuvant or locally advanced, recurrent or metastatic non-small-cell lung cancer. These indications are off-label in Ireland and require the local consent and approval process. Nivolumab combinations use a separate guide.

Other options

Discuss suitable alternative treatments, clinical trials, symptom support and the option of not proceeding. Choices depend on stage, previous treatment and your priorities.

Support at home

Use one written medication and contact plan.

Recognising side effects

Nerves and hearing

Report tingling, numbness, weakness, balance problems, ringing in the ears or hearing change early. Some effects may last after treatment.

Infusion symptoms

Tell staff immediately about flushing, rash, breathing difficulty, chest or throat tightness, or feeling faint.

Daily care

Ask for help with eating, drinking, fatigue, mouth care, hair loss or bowel symptoms. Talk about fertility, contraception and pregnancy before treatment.

Questions worth asking

Does this include immunotherapy or radiotherapy?

No. If either is prescribed, ask for the matching combination plan. A four-cycle chemotherapy-only guide cannot replace the three-cycle nivolumab plan before surgery.

When are my scans?

Your consultant agrees scan timing for your cancer and treatment aim; the NCCP regimen does not prescribe a fixed scan interval.

Who do I ring if unwell?

Keep the unit's daytime and out-of-hours numbers on your treatment summary and card. Call immediately for fever or sudden illness, even between appointments.

> **Try explaining the plan back:** Can you explain why treatment is being offered, when you attend, which medicines to take beforehand, and when and whom to call?

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1 · carboplatin and paclitaxel

Active medicines: paclitaxel + carboplatin

Emetogenic risk: high

Medicine	Dose / route	When
Aprepitant	125 mg orally on treatment day; 80 mg orally on each of the following two days	Each Day 1 chemotherapy visit
Ondansetron	16 mg orally	Each chemotherapy treatment day Review QT risk, interactions and individual dose choice.
Antiemetic dexamethasone reference	12 mg orally on treatment day; 8 mg orally daily for the next three days	Each Day 1 chemotherapy visit The team must reconcile paclitaxel allergy-prevention steroids with anti-sickness steroids and the aprepitant interaction into one prescription. Do not add courses or take leftover steroids yourself.
Metoclopramide	10 mg orally three times daily as prescribed	Subsequent days NCIS high-risk row does not define an end day or PRN qualifier; prescriber must specify duration, suitability and interaction risks.

Medicine	Dose / route	When
Paclitaxel dexamethasone premedication	Source reference 20 mg oral or intravenous (IV)	Oral approximately 6 and 12 hours before paclitaxel, or IV 30 minutes before Reconcile with antiemetic steroid; source advises considering 12 mg on treatment day when aprepitant is added. Consultant may reduce/omit only under explicit guidance.
Chlorphenamine	10 mg IV	30 minutes before paclitaxel
Famotidine	20 mg IV	30 minutes before paclitaxel Omission requires consultant guidance. Source cautions particularly with every-three-week paclitaxel; monitor closely for hypersensitivity.

Additional phase-specific support

- Take allergy-prevention and anti-sickness medicines exactly as prescribed. The team combines steroid instructions into one plan; do not add leftover tablets or miss a premedication without telling the team.
- Ask for help with poor intake, vomiting, diarrhoea or reduced urine. Follow your individual fluid advice rather than a fixed target.
- Plan transport if medicines make you drowsy. Bring your medicine list, symptom record and treatment card.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Your anatomical side-effect map

Carboplatin and paclitaxel · lung cancer

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Hair and skin

Paclitaxel

Hair loss and skin or nail changes can occur. Report severe rash or blistering urgently.

2 Mouth

Both medicines

Report painful sores or difficulty eating or drinking.

3 Blood and infection

Both medicines

Fever or sudden illness needs an immediate call, even between visits.

4 Stomach and bowels

Both medicines

Use prescribed anti-sickness medicines; call for severe pain, persistent vomiting or diarrhoea.

5 Kidneys and urine

Carboplatin

Blood tests guide the dose; report reduced urine or poor drinking.

6 Nerves and hearing

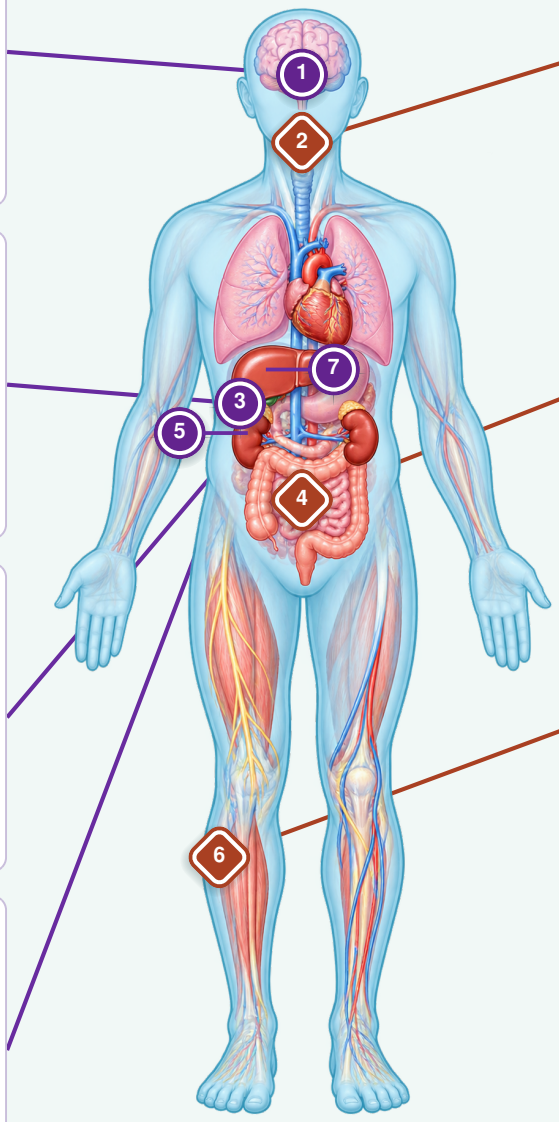
Both medicines

Report tingling, numbness, weakness, balance or hearing changes early.

7 Liver

Paclitaxel

Blood tests monitor liver function; report yellow eyes or dark urine.



Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

CHAPTER 04

Your record and contact card

IN THIS CHAPTER

Space for your own cycle or review record, questions and notes, followed by your treatment card.

Record each cycle separately. Each medicine has its own symptom table. Mark the worst symptoms since your last review, when they occurred and what they stopped you doing. Overlapping symptoms can be recorded under more than one medicine; you do not have to identify the cause.

These are patient-language severity prompts, not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) grading tool. Your team assigns the clinical grade. Serious symptoms override the columns: call without waiting to decide a severity level.

Use a new record sheet for each cycle or review. Copy or print another sheet when needed; the number of sheets does not prescribe the length of treatment.

Cycle number: _____ Start date: _____ Next review: _____

Record symptoms after each cycle and bring the record to the next review. Hair loss and other concerns can go in free-text notes.

Reusable cycle record · Day 1 · copy this sheet for another cycle of the same phase.

Paclitaxel · Chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Numbness / tingling	□ None	□ Mild change; usual tasks unaffected	□ Limits buttons, writing or walking: call	□ Washing, dressing or walking difficult: urgent review	□ Sudden major weakness / paralysis: emergency help
Fatigue	□ Usual energy	□ Tiredness improves with rest	□ Rest does not help; usual tasks affected: call	□ Self-care difficult: urgent review	□ Collapse: 112/999

Carboplatin · Chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Numbness / tingling	□ None	□ Mild change; usual tasks unaffected	□ Limits buttons, writing or walking: call	□ Washing, dressing or walking difficult: urgent review	□ Sudden major weakness / paralysis: emergency help
Bleeding / bruising	□ None	□ New small bruise / brief bleed: report	□ Repeated bleeding: call today	□ Bleeding that will not stop: urgent help	□ Heavy bleeding / collapse: 112/999
Fatigue	□ Usual energy	□ Tiredness improves with rest	□ Rest does not help; usual tasks affected: call	□ Self-care difficult: urgent review	□ Collapse: 112/999

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Carry your treatment details

Print, cut out and fold. Follow the contact plan from your own hospital.

SACTCHECK · SACT TREATMENT

Carboplatin and paclitaxel · lung cancer

NCCP 00304 v8 · 21-day cycle

Both medicines on Day 1, repeated every 21 days for four planned cycles, unless cancer progresses or side effects require a change. This standalone plan does not include nivolumab or radiotherapy.

1	2	3	4	5	6

Your contact plan

Oncology team: Insert numbers here
Out of hours: Insert numbers here.

Evenings / overnight: instructed out-of-hours pathway or nearest ED.
Emergency: 112 / 999.

Other hospital / own contact: _____

QR: https://sactcheck.com/patient/00304/ · no patient information.

Draft for review · SACTCheck v0.84.28

CHAPTER 05

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00304 v8 · official regimen**
   https://healthservice.hse.ie/documents/6924/304_V8_CARBOplatinAUC_6_PACLitaxel_200.pdf
2. **CALGB 9633 mature adjuvant analysis**
   https://pubmed.ncbi.nlm.nih.gov/18809614/
3. **ECOG 1594 advanced non-small-cell lung cancer (NSCLC) randomized comparison**
   https://pubmed.ncbi.nlm.nih.gov/11784875/
4. **NCCP NCIS antiemetics v 8**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf